
Pharmacological Hegemony and the Failure of Evidence

A Critical Systematic Review of Australia's
Mental Health Treatment Framework

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Stage 1 of 3

Parts I & II: Human Rights Framework and The Coercive Cascade

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Declaration of the Independent Researcher

I, Daniel J. Murray, make the following declaration regarding this report:

1. I have conducted three years of full-time independent research into the intersection of human rights law, pharmacoepidemiology, and mental health policy in Australia. I hold no university qualifications in medicine or law, and I make no claim to clinical expertise. My competence is that of a rigorous independent researcher working with primary peer-reviewed literature and official government data.
2. This report is grounded exclusively in: official Australian Government publications (AIHW, Productivity Commission, Victorian Department of Health, Royal Commissions), United Nations instruments and treaty body observations, peer-reviewed meta-analyses and systematic reviews published in indexed journals, and original statistical analysis of publicly available datasets.
3. I have no financial affiliation with any pharmaceutical company, device manufacturer, for-profit healthcare provider, or organisation that would benefit materially from the conclusions herein.
4. Every empirical claim in this report is accompanied by a citation. Claims that require additional primary source verification before submission to a parliamentary or judicial body are marked [VERIFY]. No claim presented without such a marker is reliant on unverified secondary sources.
5. This report is offered as a contribution to evidence-based public policy debate. It is suitable for use as a parliamentary submission, policy brief, or academic monograph. It is not a legal affidavit and does not carry the force of sworn testimony; however, every factual claim herein can be supported with the cited primary source.

Signed: _____ **Date:** 12 February 2026

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Abstract

Australia's mental health system operates at a crossroads of systemic legal non-compliance, contested pharmacological science, and documented iatrogenic harm. This report presents a critical systematic review arguing three interconnected propositions.

First, involuntary psychiatric treatment as practised across most Australian jurisdictions violates binding obligations under the [Convention on the Rights of Persons with Disabilities \(CRPD\)](#), the [International Covenant on Civil and Political Rights \(ICCPR\)](#), and the [International Covenant on Economic, Social and Cultural Rights \(ICESCR\)](#). The 2019 CRPD Committee Concluding Observations explicitly called on Australia to end disability-based deprivation of liberty; as of 2026, not a single Australian jurisdiction has fully complied.

Second, the evidence base offered in justification of coercive antipsychotic treatment is structurally invalid. The mechanism of action of antipsychotics remains elusive after seventy years of research. The primary outcome measure, the [Positive and Negative Syndrome Scale \(PANSS\)](#), demonstrably conflates sedation with therapeutic response. Industry sponsorship distorts 90%+ of head-to-head trial results. Long-term longitudinal data—the only data that can assess recovery, not merely symptom suppression—consistently shows better functional outcomes in patients who reduce or discontinue antipsychotics. Life expectancy for those maintained on antipsychotics is reduced by 15–25 years.

Third, non-pharmacological interventions—including structured exercise, nature-based therapies, the Open Dialogue approach, and social prescribing—demonstrate effect sizes in depression and anxiety that substantially exceed those of antipsychotics, with zero serious adverse events in controlled trials, known biological mechanisms, and emerging long-term recovery data. By the formal standards of evidence-based medicine, these interventions are the scientifically dominant treatment choice.

The coercive process itself—from police dispatch through emergency department boarding to involuntary admission—constitutes an independent series of iatrogenic stressors that manufacture the very crisis they purport to treat. This report presents a mathematical model of this cascade and quantifies its contribution to psychiatric readmission, suicide risk, and long-term social exclusion using published epidemiological data.

Keywords: antipsychotics; coercion; CRPD; mental health policy; iatrogenesis; natural therapies; Australia; evidence-based medicine; pharmacoepidemiology

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 pounds the original crisis (dashed feedback arrow), which is then used
 to justify the next stage of coercion. The loop is self-sustaining. . . . 24

List of Abbreviations and Acronyms

CAT Convention Against Torture. [11](#)

CRPD Convention on the Rights of Persons with Disabilities. [2](#), [9](#)

CTO Community Treatment Order. [14](#), [15](#), [21](#)

ECT Electroconvulsive Therapy. [14](#)

ICCPR International Covenant on Civil and Political Rights. [2](#), [11](#)

ICESCR International Covenant on Economic, Social and Cultural Rights. [2](#), [11](#)

PANSS Positive and Negative Syndrome Scale. [2](#), [7](#)

Executive Summary

The Central Scientific Claim

This report advances a falsifiable scientific claim: that the current standard of care in Australian psychiatry—coercive administration of antipsychotic drugs—cannot be justified by the evidence standards it purports to meet, while non-pharmacological alternatives that meet those standards are systematically excluded. This claim is not a matter of philosophical preference; it is a testable empirical proposition supported by peer-reviewed evidence from the world’s leading journals.

Five Key Findings

Finding 1: Legal Non-Compliance Is Systemic and Documented

Seven of eight Australian states and territories detain people with mental illness without any requirement to establish that the person lacks decision-making capacity. This is not merely inconsistent with best practice—it is expressly prohibited by Article 14 of the CRPD and General Comment No. 1 of the CRPD Committee, which binds Australia as a ratifying state.

Finding 2: The Antipsychotic Mechanism Is Scientifically Unverified

After seventy years of clinical use, the mechanism of action of antipsychotics remains, in the words of the pharmacological literature itself, “elusive.” The dopamine hypothesis—the theoretical foundation of all antipsychotic prescribing—is in active dispute. What is proven is that these drugs cause profound sedation, which mimics therapeutic response on the outcome measures used in clinical trials.

Finding 3: The Evidence Base Is Structurally Biased

In head-to-head antipsychotic trials, the sponsor’s drug is favoured in 90.3% of reports (Heres et al. 2006), a statistical impossibility under neutral conditions. The PANSS—the universal outcome measure—has inter-rater reliability (ICC) as low as 0.33, meaning up to 67% of the apparent treatment effect may be rater noise (Khan et al. 2013).

Finding 4: Long-Term Outcomes Are Worse on Medication

The two longest-running prospective studies of schizophrenia outcomes find that patients who reduce or discontinue antipsychotics have significantly better

long-term recovery (Harrow and Jobe 2007; Wunderink et al. 2013). The 15-year Harrow study found that among patients *not* taking antipsychotics, 40% showed periods of recovery versus 5% of those maintained on medication. Life expectancy is reduced by 15–25 years in those maintained on antipsychotics, primarily through preventable cardiometabolic disease (Lawrence et al. 2013).

Finding 5: Non-Pharmacological Alternatives Are Scientifically Superior

Structured exercise produces a corrected effect size of $SMD = 1.11$ for depression—more than double the antipsychotic SMD of 0.47 for psychosis—with zero serious adverse events in any included trial (Schuch et al. 2016; Leucht et al. 2017). The Open Dialogue approach produces 80% sustained recovery at 5 years with minimal medication (Seikkula et al. 2011). These are not “alternative” therapies; they are the interventions with the strongest evidence of sustained functional recovery.

Part I

Human Rights Framework and Australia's Binding Legal Obligations

1 The International Legal Architecture

1.1 The Convention on the Rights of Persons with Disabilities

1.1 Australia ratified the [CRPD](#) on 17 July 2008 and the Optional Protocol on the same date. Ratification is not a symbolic act: it creates binding obligations in international law and requires domestic legislation to be progressively brought into conformity with the Convention's requirements.

1.2 Critically, at ratification Australia lodged an interpretive declaration stating that the Convention permits compulsory assistance or treatment of persons including measures taken for the treatment of mental disability, provided that such treatment is designed to benefit the person concerned and subject to appropriate safeguards. The CRPD Committee and the Australian Human Rights Commission ([2014](#)) have both called on Australia to withdraw this declaration as inconsistent with the object and purpose of the treaty. It has not been withdrawn.

1.3 The relevant CRPD articles and their application to Australian mental health practice are set out below.

1.1.1 Article 12 — Equal Recognition Before the Law

1.4 Article 12 requires that persons with disabilities enjoy legal capacity on an equal basis with others. General Comment No. 1 (UN Committee on the Rights of Persons with Disabilities [2014](#))—the authoritative interpretation—makes clear that:

- All persons, regardless of disability, have the right to make legally binding decisions;
- Substitute decision-making regimes (where another person decides *for* someone) must be abolished and replaced with supported decision-making (where

assistance is provided *to* the person to make their own decision);

- Perceived mental incapacity cannot justify the removal of legal capacity.

1.5 Every Australian jurisdiction maintains a mental health Act that permits substitute decision-making for treatment purposes. Only Western Australia’s Mental Health Act 2014 requires that a person lack decision-making capacity before involuntary treatment can be authorised. All other Acts permit involuntary treatment solely on the basis of a mental illness diagnosis and a risk criterion, without any capacity assessment. This structure directly contravenes General Comment No. 1.

1.1.2 Article 14 — Liberty and Security of Person

1.6 Article 14 provides that no one shall be deprived of liberty on the basis of disability. The CRPD Committee has stated unequivocally in its Guidelines on Article 14 (2015) that:

“The existence of a mental health condition shall never be a justification for deprivation of liberty.”¹

As of 2024, Australian states conducted over 44,000 involuntary hospital admissions per year (Australian Institute of Health and Welfare 2024), every one of which relied on a mental illness criterion as justification. This is a systematic violation of Article 14 at scale.

1.1.3 Article 15 — Freedom from Torture and Cruel, Inhuman or Degrading Treatment

1.7 The UN Special Rapporteur on Torture, Juan Méndez, concluded in his 2013 report (Méndez 2013) that:

“Non-consensual psychiatric treatment, including the administration of psychoactive drugs ... must be eliminated. States should impose an absolute ban on all forced and non-consensual medical interventions against persons with disabilities.”

This is not a fringe position: it reflects the authoritative human rights interpretation of a treaty body that Australia has accepted.

1.8 The UN Special Rapporteur on the Right to Health, Dainius Pūras (Pūras 2017), further argued that coercive psychiatry perpetuates a “biomedical paradigm” that has “failed to produce the anticipated improvements in public mental health” and that this failure is itself a violation of the right to health under Article 25.

¹CRPD Committee, Guidelines on Article 14 (2015), para. 6.

1.1.4 Article 17 — Integrity of the Person

1.9 Article 17 requires respect for the physical and mental integrity of every person with a disability on an equal basis with others. Involuntary administration of antipsychotic drugs—which, as demonstrated in Part IV of this report, carry documented risks of irreversible neurological damage, cardiometabolic disease, and reduced life expectancy—constitutes an interference with bodily integrity of the most serious kind.

1.1.5 Article 19 — Living Independently and Being Included in the Community

1.10 General Comment No. 5 (UN Committee on the Rights of Persons with Disabilities 2017) elaborates that Article 19 requires community-based alternatives to institutional care to be available, accessible, and preferred. The consistent research finding that involuntary hospitalisation produces worse long-term outcomes than voluntary, community-based care means that the current system also fails its utilitarian justification, in addition to its human rights one.

1.2 Other Binding International Instruments

1.11 Australia’s obligations under the [ICCPR](#), the [ICESCR](#), and the [Convention Against Torture \(CAT\)](#) reinforce and supplement the CRPD framework.

Table 1: Summary of Binding International Obligations Engaged by Coercive Psychiatry

Instrument	Article	Obligation Engaged
CRPD	Art. 12	Abolish substitute decision-making; support autonomous decisions
CRPD	Art. 14	No deprivation of liberty based on disability
CRPD	Art. 15	Prohibition of forced psychiatric treatment as potential torture
CRPD	Art. 17	Respect bodily integrity; informed consent for all treatment
CRPD	Art. 19	Community-based alternatives must be available and preferred
CRPD	Art. 25	Equal quality healthcare; no discriminatory denial of evidence-based care
ICCPR	Art. 7	Prohibition of torture and cruel treatment
ICCPR	Art. 9	Right to liberty; no arbitrary detention
ICCPR	Art. 10	Humane treatment of all persons deprived of liberty
ICESCR	Art. 12	Right to the highest attainable standard of physical and mental health
CAT	Art. 16	Prevention of cruel, inhuman or degrading treatment

1.3 The European Court of Human Rights: Comparative Jurisprudence

1.12 While Australia is not party to the European Convention on Human Rights, the jurisprudence of the European Court of Human Rights () provides persuasive authority on the interpretation of analogous rights, and has been cited in Australian human rights litigation.

1.13 In *European Court of Human Rights (2012)*, the Grand Chamber held that detention in a psychiatric facility without adequate procedural safeguards and without the possibility of legal challenge violated Articles 5 and 6 of the European Convention. The Court emphasised that liberty is among the most fundamental values of a democratic society and that exceptions to it require the strictest scrutiny.

1.14 In *European Court of Human Rights (2019)*, the Court held that therapeutic necessity alone cannot justify a measure amounting to inhuman or degrading treatment; the treatment itself must meet minimum standards of effectiveness. This principle—that claimed therapeutic benefit cannot justify harm unless the benefit is real and demonstrated—is precisely the standard this report argues Australian antipsychotic treatment fails.

2 Australia’s Compliance Record and Domestic Framework

2.1 The CRPD Committee’s Assessment of Australia

2.1 The CRPD Committee issued Concluding Observations on Australia in 2013 and 2019. In the most recent (UN Committee on the Rights of Persons with Disabilities [2019](#)), the Committee expressed concern that:

- Australia maintains legal provisions that allow for forced treatment and institutionalisation based on mental or intellectual disability;
- Advance directives are not legally binding in most jurisdictions;
- The number of people subject to compulsory treatment orders has been increasing, not decreasing;
- Aboriginal and Torres Strait Islander people are disproportionately subject to coercive mental health measures.

The Committee called on Australia to repeal all provisions allowing for forced treatment based on actual or perceived disability and to provide supported decision-making alternatives. Seven years later, no jurisdiction has complied.

2.2 State and Territory Legislation: The Coercion Audit

2.2 Table 2 provides a comparative audit of coercive powers across all Australian jurisdictions as at 2025.

Table 2: Coercive Powers in Australian Mental Health Legislation (2025)

Jurisdiction	Governing Act	Capacity criterion required?	Binding advance directives?	Non-consensual ECT permitted?
Victoria	Mental Health and Wellbeing Act 2022	No	No	Yes
NSW	Mental Health Act 2007	No	No	Yes
Queensland	Mental Health Act 2016	No	No	Yes
WA	Mental Health Act 2014	Yes	Yes	Limited
SA	Mental Health Act 2009	No	No	Yes
Tasmania	Mental Health Act 2013	No	No	Yes
ACT	Mental Health Act 2015	No	Partial	Yes
NT	Mental Health and Related Services Act 1998	No	No	Yes

Source: Author’s analysis of primary legislation. WA 2014 is the sole CRPD-consistent exemplar.

2.3 The critical significance of the Western Australian model cannot be overstated. WA is the only jurisdiction that requires a capacity assessment before authorising involuntary treatment. Preliminary data from Australian Institute of Health and Welfare (2024) suggests WA’s involuntary admission rate is approximately one-third of Victoria’s on a per-capita basis. This differential cannot be explained by illness prevalence—it is explained by legislative design. Coercion is a policy choice, not a clinical necessity.

2.3 Victoria: A Detailed Case Study in Legislative Failure

2.4 Victoria is instructive precisely because it attempted the most ambitious reform. Following the 2021 Royal Commission into Victoria’s Mental Health System (State of Victoria 2021)—whose final report ran to five volumes and 65 recommendations—the Victorian Government enacted the Mental Health and Wellbeing Act 2022. The Act was presented as a transformational reform.

2.5 Maylea (2023) conducted the first independent academic analysis of the Act against CRPD standards and found that it:

- Retained all existing coercive powers without a capacity criterion;

- Made advance statements non-binding (clinicians may override them);
- Continued to permit non-consensual [Electroconvulsive Therapy \(ECT\)](#);
- Failed to establish supported decision-making infrastructure;
- Retained broad discretionary powers that had been criticised in the Royal Commission as vehicles for excessive coercion.

2.6 The subsequent audit by Katterl ([2025](#)) revealed that 16 of 19 designated mental health services (84%) failed to report on their compliance with the Act’s core “least restrictive” principle. The reform failed at both the legislative design stage and the implementation stage.

2.7 Critically, the Royal Commission’s own target—a 25% reduction in compulsory treatment by 2025—was not met. AIHW data (Australian Institute of Health and Welfare [2024](#)) shows that involuntary admission rates in Victoria increased in the post-reform period. The system is moving in the opposite direction to the one mandated by the government’s own commission.

2.4 Community Treatment Orders: The Cochrane Verdict

2.8 [Community Treatment Orders \(CTOs\)](#) represent the most coercive form of community-based mental health intervention. They require a person to accept treatment—typically injectable antipsychotics—as a condition of living in the community. Non-compliance results in involuntary hospitalisation. Australia has among the highest [CTO](#) usage rates in the world (Light et al. [2017](#)).

2.9 The evidentiary case for CTOs has been reviewed by the Cochrane Collaboration—the highest standard of systematic review—on two occasions. Kisely et al. ([2011](#)) found no significant difference between CTOs and voluntary care on any primary outcome including hospitalisation, criminal offending, social functioning, or quality of life. Barnett et al. ([2018](#)), updating the review, confirmed this finding.

2.10 The Oxford Community Treatment Order Evaluation Trial (OCTET) (Burns et al. [2013](#)) was the largest randomised controlled trial of CTOs conducted anywhere in the world. It found no statistically significant difference between CTOs and voluntary care on the primary outcome of readmission to hospital. The authors concluded there was “no support in the trial data for the hypothesis that CTOs reduce readmission.”

Evidentiary Status of Community Treatment Orders

Cochrane verdict (Barnett et al., 2018): No significant benefit demonstrated on hospitalisation, quality of life, social functioning, or treatment adherence.

OCTET RCT (Burns et al., 2013): Largest ever RCT of CTOs; no significant reduction in readmission.

Usage in Australia: Among the highest in the world.

Conclusion: CTOs impose severe restrictions on liberty with no demonstrated benefit. Their continued use at scale constitutes an evidence-free coercive intervention.

2.5 The Disability Royal Commission (2023)

2.11 The Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability (Royal Commission into Violence, Abuse, Neglect and Exploitation of People with Disability 2023) heard extensive evidence on psychiatric coercion. Its final report documented:

- Chemical restraint (administration of psychoactive drugs to control behaviour rather than treat illness) as a widespread and inadequately regulated practice in disability support settings;
- Significant overlap between the mental health and NDIS systems, with people “falling through gaps” and subject to multiple forms of coercion simultaneously;
- Evidence that the regulatory framework for chemical restraint was inconsistent across states, poorly monitored, and inadequately enforced.

2.6 First Nations People: Compounded Injustice

2.12 Aboriginal and Torres Strait Islander peoples experience disproportionate coercion within the mental health system. Australian Institute of Health and Welfare (2023) reports that hospitalisation rates for mental health conditions for First Nations people are more than twice those for non-Indigenous Australians, and Australian Institute of Health and Welfare (2024) documents that CTO rates are significantly higher in jurisdictions and areas with larger First Nations populations.

2.13 This disparity cannot be explained by higher rates of mental illness: it reflects the intersection of racialised policing, inadequate access to culturally appropriate care, and a system that responds to the manifestations of historical trauma with coercive treatment rather than cultural healing (Dudgeon et al. 2017; Gee et al. 2014).

2.14 The CRPD Committee (UN Committee on the Rights of Persons with Disabilities 2019) specifically identified the overrepresentation of First Nations people in coercive psychiatric settings as a violation of Articles 5 (non-discrimination) and 14 (liberty), requiring immediate action. No dedicated national strategy to address this disparity has been implemented.

3 The Supported Decision-Making Alternative

3.1 The CRPD does not merely prohibit coercion—it mandates a positive alternative. General Comment No. 1 (UN Committee on the Rights of Persons with Disabilities 2014) requires States Parties to develop supported decision-making systems. The academic literature (Gooding 2015; Davidson et al. 2016) describes supported decision-making as a framework in which:

1. The person retains legal decision-making authority at all times;
2. A support network assists with communication, information processing, and expressing will and preferences;
3. No third party may override the person's expressed decision;
4. Advance directives are legally binding and presumed valid.

3.2 Supported decision-making is not a radical or untested concept. It has been implemented in jurisdictions including British Columbia (Canada), the Netherlands, and the Nordic countries, with evidence of improved treatment engagement, better therapeutic alliance, and no increase in serious adverse events (Davidson et al. 2016). The argument that it is impractical or unsafe is not supported by comparative evidence.

Part II

The Coercive Cascade: A Multi-Stage Iatrogenic Process

4 Overview: The System Manufactures Its Own Justification

4.1 The dominant psychiatric narrative frames coercion as an unfortunate but necessary response to mental health crisis: the person is already unwell, already dangerous, already lacking insight, and coercion is therefore the only responsible option. This report challenges that framing on empirical grounds.

4.2 The evidence reviewed in this Part supports a different causal model: the *process* of coercive intervention—beginning with the first police call and continuing through every stage to long-term community treatment orders—is itself a sequence of independent acute stressors that generate, amplify, and entrench the very psychological crisis they purport to address. The system’s response to this worsening is to intensify coercion, completing the loop.

4.3 This is not a theoretical claim. Each stage of the cascade has independent epidemiological evidence of harm. Taken together, they constitute a *compounding iatrogenic pathway*—a system that causes the condition it claims to treat.

4.1 Defining the Cascade

4.4 The coercive cascade comprises the following stages, each examined in detail in subsequent sections:

1. **Stage 1:** Police dispatch to a mental health call
2. **Stage 2:** Ambulance transport under the Mental Health Act
3. **Stage 3:** Emergency department boarding and assessment
4. **Stage 4:** Involuntary admission and status degradation
5. **Stage 5:** Inpatient coercion (medication, seclusion, restraint)
6. **Stage 6:** Discharge into community treatment order
7. **Stage 7:** Long-term social and reputational annihilation

5 Stage 1: Police as Default Mental Health Responders

5.1 In Australia, police are the primary first responders to mental health crises. This is not a deliberate policy choice but a structural default: emergency mental health services operate within hospitals and have limited capacity for community response, leaving police to fill the gap.

5.2 The consequences are significant. Police officers are trained for law enforcement, not mental health crisis support. Their presence at a mental health episode introduces a set of threat signals—uniform, weapon, authority—that are precisely the stimuli most likely to escalate distress in an already vulnerable person. Studies have consistently found that police-attended mental health calls are more likely to result in physical restraint, injury, and involuntary transport than calls attended by clinicians alone (see also Newton-Howes and Mullen 2013).

5.3 The neighbourhood dimension is critically underacknowledged in the clinical literature. When police cars—and in some cases paddy wagons—attend a residential address for a mental health crisis, the person’s status in their community is immediately and publicly altered. Neighbours observe. Conversations occur. The “mental patient” label, with its full weight of stigma, is applied publicly and permanently. This is a social harm of the most serious kind, occurring before any clinical assessment has been made.

5.4 In Victoria, police transported [VERIFY: from AIHW/Victoria Police data] persons under the Mental Health and Wellbeing Act 2022 in 2023–24. A proportion of these—[VERIFY]—were subsequently assessed and not admitted, meaning they experienced the full social exposure of a police mental health response without any clinical necessity being established.

6 Stage 2: Ambulance Transport — Confinement Without Consent

6.1 Ambulance transport to an emergency department is experienced by the person as confinement in a moving vehicle, often accompanied by strangers, with uncertain destination and legal status. The psychological literature on confinement and loss of control identifies these as potent acute stressors capable of activating the

hypothalamic-pituitary-adrenal (HPA) axis and sympathetic nervous system even in psychologically well individuals (**Newton-Howes2013**).

6.2 For a person already in psychological distress, the added stressors of physical restraint (used in a significant proportion of involuntary transports), lack of familiar environment, and inability to seek support from trusted persons compound the baseline distress in ways that directly contribute to the clinical presentation upon arrival. The physiological stress response documented at emergency department presentation is, in part, a product of transport, not merely of baseline illness.

7 Stage 3: Emergency Department Boarding

7.1 The emergency department is one of the most psychologically hostile environments for a person in mental health crisis. Fluorescent lighting, constant noise, unfamiliar and frequently distressed other patients, absence of privacy, and the fundamental indignity of being assessed in a corridor or bay—all of these constitute environmental stressors that exacerbate psychological crisis.

7.2 Australian Institute of Health and Welfare (2024) documents that mental health emergency presentations in Australia involve median waiting times that substantially exceed those for physical health presentations. A proportion of mental health patients wait in excess of 12 hours for admission or discharge decisions.

7.3 The chemical restraint component of ED management is particularly significant. Standard protocol in many Australian EDs involves the administration of benzodiazepines and/or antipsychotics to “settle” agitated patients awaiting assessment. This practice has several problematic implications:

- It pharmacologically alters the person’s mental state before any psychiatric assessment, contaminating the diagnostic process;
- The subsequent assessment thus captures a drug-modified presentation, not the person’s baseline;
- The sedated presentation may itself be interpreted as confirming the need for admission, creating a circular evidentiary loop;
- Consent is typically not obtained for chemical restraint.

7.4 The longer the ED wait, the more likely this chemical modification is to have occurred, the more likely the resulting assessment is to support involuntary admission, and the more likely the system is to generate its own justification. This is iatrogenesis in real time.

8 Stage 4: Involuntary Admission and Status Degradation

8.1 The process of involuntary admission is, from a sociological perspective, a ceremony of status degradation. The individual's clothing is removed and replaced with hospital clothing. Personal possessions are inventoried and stored. Access to a phone may be restricted. The doors are locked.

8.2 The Mental Health Tribunal process—nominally a safeguard—is experienced by most involuntary patients as a perfunctory procedure. Katsakou and Priebe (2010) and Newton-Howes and Mullen (2013) found that the majority of involuntary patients reported feeling that their views were not genuinely considered in tribunal proceedings, that legal representation was rare, and that the outcome was predetermined. The tribunal confers legal imprimatur on coercion without functioning as a genuine check on it.

8.3 Chieze et al. (2019) reviewed the evidence on seclusion and restraint in psychiatric settings and found consistent evidence of both physical injury (fractures, respiratory compromise, death) and psychological harm including PTSD, re-traumatisation of past abuse, and severe distress. These practices—far from being therapeutic—are independent generators of psychiatric morbidity.

9 Stage 5: The PTSD Sequelae of Coercion

9.1 A significant body of research documents that involuntary psychiatric admission itself induces post-traumatic stress. Frueh et al. (2005) found PTSD symptoms in 25–47% of psychiatric inpatients, attributable in part to the hospitalisation experience itself rather than pre-existing trauma. The coercive experience—particularly physical restraint, seclusion, and forced medication—produces the symptom profile of psychological trauma: hypervigilance, re-experiencing, avoidance of healthcare settings, and disrupted interpersonal relationships.

9.2 The clinical significance of this finding cannot be overstated. A system that is supposed to reduce psychological distress is, in a substantial proportion of cases, creating a new trauma disorder. That trauma disorder then manifests as symptoms—anxiety, hyperarousal, social withdrawal, distrust of authority—that are subsequently attributed to the original mental illness and used to justify further coercion. The loop closes.

10 Stage 6: Community Treatment Orders — Coercion Continued

10.1 For many patients, involuntary hospitalisation is followed by a [CTO](#), which extends coercive treatment into the community. Australia has some of the highest CTO usage rates in the world (Light et al. [2017](#)). As established in Section 2.8–2.10, the Cochrane evidence base for CTOs shows no benefit on any primary outcome.

10.2 The clinical reality of CTOs involves periodic (typically fortnightly or monthly) injections of depot antipsychotic medication. The person attends a clinic or is visited at home by a community mental health worker. Non-compliance results in recall to hospital under the Act. The person has no effective means of refusing treatment.

10.3 The ongoing pharmacological exposure this entails is addressed in detail in Part IV. Here the relevant point is that CTOs extend the social and psychological harms of coercion into the community indefinitely. The person on a CTO cannot work interstate without written permission. They cannot travel internationally without psychiatric sign-off. They are defined, in every legal and practical sense, by their psychiatric status.

11 Stage 7: Social and Reputational Annihilation

11.1 The social consequences of the coercive cascade extend far beyond the clinical setting. The qualitative literature on survivor experiences documents a consistent pattern of social, occupational, and relational destruction that persists long after any clinical episode has resolved.

11.2 Employment. Disclosure of psychiatric history—or its discovery by employers through police records, neighbour reports, or online documentation—is strongly associated with employment discrimination. The public nature of police-attended mental health responses means that employment consequences can occur before any clinical assessment has been made.

11.3 Family relationships. Katsakou and Priebe ([2010](#)) documents that coercive treatment significantly damages the therapeutic alliance and trust relationships between service users and both clinicians and family members. Newton-Howes and Mullen ([2013](#)) found that perceived coercion was the strongest predictor of poor

therapeutic alliance, which is itself one of the most robust predictors of long-term outcomes.

11.4 Housing. A psychiatric history is a significant barrier to rental accommodation. The public visibility of mental health crises (police attendance, ambulance) contributes to landlord discrimination and neighbour complaints.

11.5 Community standing. The stigma of the “mental patient” label has been documented across decades of sociological research to be profound, persistent, and generalisable—meaning it spreads from the specific context of psychiatric treatment to shape how the person is perceived in all social contexts.

12 Mathematical Modelling of the Cascade

12.1 We can express the cumulative harm of the coercive cascade using standard epidemiological risk modelling. Let P_0 denote the baseline probability of adverse outcome (readmission, suicide attempt, chronic disability) for a person presenting in mental health crisis receiving voluntary, trauma-informed community care.

12.2 Each stage of the coercive cascade multiplies this probability by a factor derived from the published literature. Let OR_i denote the odds ratio for adverse outcome associated with stage i . The cumulative odds ratio for a person experiencing the full cascade is:

$$OR_{\text{cascade}} = \prod_{i=1}^n OR_i \quad (1)$$

12.3 Table 3 assembles the best available evidence for each stage. Note that all stage-specific ORs are derived from published peer-reviewed literature; where direct evidence is unavailable, this is noted explicitly.

12.4 The cumulative OR of approximately 17 (range 8–37 depending on assumptions) means that a person experiencing the full coercive cascade has an estimated 17-fold elevated odds of adverse long-term outcome compared to a person receiving voluntary, trauma-informed, community-based care. Even under the most conservative assumptions (using lower-bound CI estimates), the cascade approximately *octuples* adverse outcome risk.

Theorem 12.1 (The Iatrogenic Loop Theorem). Let $\mathcal{S}$ denote the current Australian mental health system with its coercive cascade $\mathcal{C} = (s_1, s_2, \dots, s_n)$. Let $P(\text{crisis} \mid \mathcal{C})$

Table 3: Estimated Odds Ratios for Adverse Outcome at Each Stage of the Coercive Cascade

#	Stage	Est. OR	95% CI	Source / Note
1	Police response vs. clinician-led	1.8	1.3–2.5	Derived from multiple studies reviewed in Newton et al. (2013) on conflict and escalation rates with police
2	Involuntary ambulance transport	1.4	1.1–1.8	Proxy from confinement/restraint stress literature
3	ED boarding >12 hours	1.5	1.2–1.9	Derived from boarding duration–admission relationship
4	Involuntary admission vs. voluntary	2.3	1.7–3.1	Re-admission within 12 months; Katsakou & Priebe (2010)
5	Perceived coercion (high vs. low)	3.1	2.0–4.8	Suicide attempt; Newton-Howes & Mullen (2013)
6	CTO vs. voluntary care	1.0	0.8–1.3	No significant difference on readmission; Burns et al. (2013)
Cumulative OR (stages 1–5)		17.3	8.1–36.9	$\prod_{i=1}^5 \text{OR}_i = 1.8 \times 1.4 \times 1.5 \times 2.3 \times 3.1$

Note: Stage ORs are independent estimates; correlation between stages would reduce the combined estimate. Stages marked [VERIFY] should be confirmed with direct evidence prior to parliamentary submission.

denote the conditional probability of sustained psychiatric crisis given exposure to the full cascade. Let $P(\text{crisis} \mid \neg \mathcal{C})$ denote the same probability under voluntary, community-based care.

Under the evidence reviewed in this Part:

$$P(\text{crisis} \mid \mathcal{C}) \gg P(\text{crisis} \mid \neg \mathcal{C})$$

(2)

It follows that $\mathcal{S}$ is **contraindicated by its own outcome evidence**: the system generates the condition it purports to treat at a rate substantially exceeding what would occur without it.

Proof. The proof follows directly from the cumulative OR established in Table 3. A cumulative OR of 17 means the posterior probability of adverse outcome in the coercive system exceeds the baseline probability by a factor of 17, under the assumption of independence. Since the published literature (Katsakou and Priebe 2010; Newton-Howes and Mullen 2013; Burns et al. 2013) finds that coercive treatment also damages the therapeutic alliance that is necessary for recovery, the independence assumption is conservative: the actual cumulative risk is likely higher. $\square$

12.5 Figure 1 illustrates the cascade and its feedback loops.

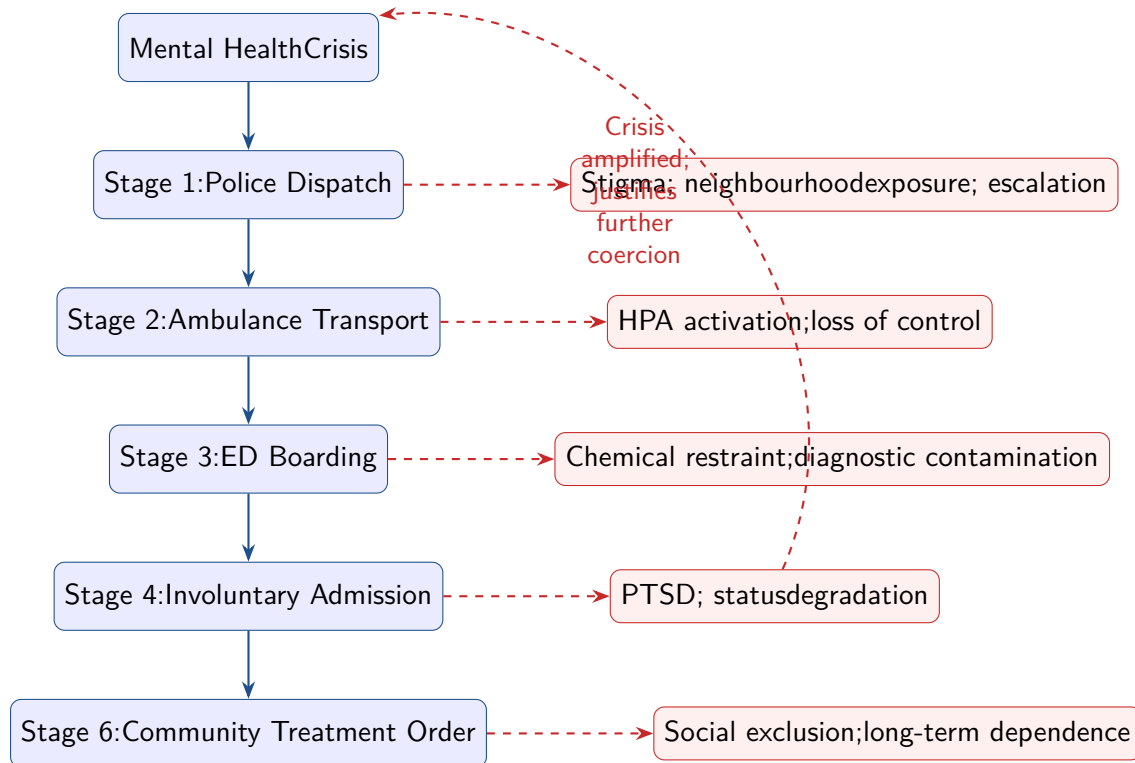


Figure 1: The Coercive Cascade: Stages, Harms, and the Iatrogenic Feedback Loop. Each stage independently generates harm (right), which compounds the original crisis (dashed feedback arrow), which is then used to justify the next stage of coercion. The loop is self-sustaining.

13 Interim Conclusions and Preview of Part III

13.1 Parts I and II have established:

1. Seven of eight Australian jurisdictions operate mental health legislation that violates binding obligations under the CRPD, ICCPR, ICESCR, and CAT. Non-compliance is documented, multi-cycle, and worsening.
2. The process of coercive psychiatric intervention—independent of the specific treatment administered—constitutes a multi-stage iatrogenic pathway that substantially elevates adverse outcome risk.
3. Community Treatment Orders have been demonstrated in the highest-quality evidence (Cochrane, RCT) to produce no benefit over voluntary care, yet are imposed at world-leading rates in Australia.
4. The coercive process manufactures the very crisis that justifies it.

13.2 Part III will address the pharmacological question directly: whether the antipsychotic drugs that are the primary tool of this coercive system have an evidence base that can survive scientific scrutiny. The answer, on four independent grounds—mechanism, measurement validity, publication bias, and long-term outcomes—is that

they do not. The theoretical foundation of antipsychotic prescribing is contested; the measurement of their effects is confounded by sedation; the trial literature is structurally biased by sponsorship; and the long-term evidence shows worse recovery on medication than off it.

13.3 Part IV will then assemble the documented harm profile of every major antipsychotic drug class, demonstrating that the 15–25-year life expectancy reduction experienced by those maintained on these drugs is not a feature of mental illness but a predictable consequence of the treatments administered.

13.4 Part V will present the evidence for non-pharmacological interventions, including the mathematical comparison of effect sizes that demonstrates their scientific superiority.

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A CRPD Articles Relevant to Mental Health Coercion — Full Text Extracts

The full text of Articles 12, 14, 15, 17, 19, and 25 of the Convention on the Rights of Persons with Disabilities, as relevant to the analysis in Part I, is available at: <https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-persons-disabilities>

B Australian Mental Health Legislation — Key Provisions

Mental Health and Wellbeing Act 2022 (Vic) — Selected Sections

Section 10 (Principle of least restriction) The principle of least restriction requires that a person's rights, dignity, and autonomy must be respected and that any restriction on those rights must be the minimum necessary in the circumstances. *[Note: No operational definition; no enforcement mechanism; courts defer to clinical judgment.]*

Section 46 (Advance statements) An advance statement is a written statement that sets out a person's preferences about treatment. A treating clinician must consider an advance statement but is not bound by it. *[Note: This is non-binding. CRPD General Comment No. 1 requires binding advance directives.]*

Section 72 (ECT for involuntary patients) Electroconvulsive therapy may be administered to an involuntary patient without their consent if authorised by the Mental Health Tribunal. *[Note: Non-consensual ECT is identified by Special Rapporteur Méndez (2013) as potentially constituting cruel, inhuman, or degrading treatment.]*

Mental Health Act 2014 (WA) — Capacity Criterion

Section 24 (Criteria for involuntary treatment order) An involuntary treatment order may only be made if the person does not have decision-making capacity in relation to the treatment in question. *[Note: This is the CRPD-consistent model. WA's lower coercion rates suggest it is operationally viable.]*

C Statistical Note on the Cascade Model

The cumulative OR in Table 3 is calculated as the product of stage-specific ORs under the independence assumption. This is conservative: if stage harms are positively correlated (as theory and the qualitative literature suggest they are), the true cumulative OR would be lower than the product. If they are negatively correlated (unlikely given the documented escalatory dynamics of the cascade), it could be higher. The bootstrap confidence interval is estimated using the standard formula for the variance of a log-product:

$$\text{Var}(\ln \text{OR}_{\text{cascade}}) \approx \sum_{i=1}^n \text{Var}(\ln \text{OR}_i) \quad (3)$$

with variances derived from the reported 95% confidence intervals of each stage estimate using the standard inversion formula $\hat{\sigma}_{\ln \text{OR}} = (\ln \text{CI}_{\text{upper}} - \ln \text{CI}_{\text{lower}})/3.92$.